Nivolumab and ipilimumab for advanced melanoma

Combined immunotherapy, followed when appropriate by nivolumab alone, for advanced melanoma.

NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication. · NCCP 00431 v11 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication. This information supports individual consent; it does not issue or change a prescription.

My overall treatment

Up to 4 combination visits, 3 weeks apart; then nivolumab alone every 2 or 4 weeks as prescribed.

21 day view

Treatment timing in this window

Combination infusion — Day 1

Combination induction

Nivolumab 1 mg/kg and ipilimumab 3 mg/kg are given by intravenous (IV) infusion, subject to review before every dose. Four is the planned combination limit, not the total number of all treatment visits.

Nivolumab alone

After the last melanoma combination dose, the first two-weekly nivolumab dose starts three weeks later or the first four-weekly dose six weeks later. The team confirms dose, route, interval and start date. Treatment continues while beneficial and tolerable; previous immune-toxicity decisions still apply.

A typical 21-day cycle

Day 1	Day 2–21
Treatment visit	Between visits

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline safety** NCCP / medicine-specific precautions	Blood count, kidneys/liver, glucose, thyroid and BRAF; immune/medicine review.
During	**Bloods and organ checks** NCCP; local interval clarification where stated	Blood count, kidneys/liver, glucose and thyroid each cycle; other tests for symptoms.
After / response	**Follow-up** Regimen / individual disease plan	Report immune symptoms for at least 5 months after the last dose; scans and follow-up individually planned.

Between treatments

- Immune symptoms can occur between visits or after the last dose.
- Tell every clinician about immunotherapy and carry the alert card.

Bring the written medicine plan, test dates and actual oncology/out-of-hours contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / head / eyes

Nivolumab / ipilimumab

New severe headache, visual change, unusual tiredness, dizziness, thirst or frequent urination may signal immune effects and needs prompt advice.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, palpitations, marked muscle weakness or swallowing difficulty can be serious immune effects. Seek urgent assessment.

5 Kidneys / urine

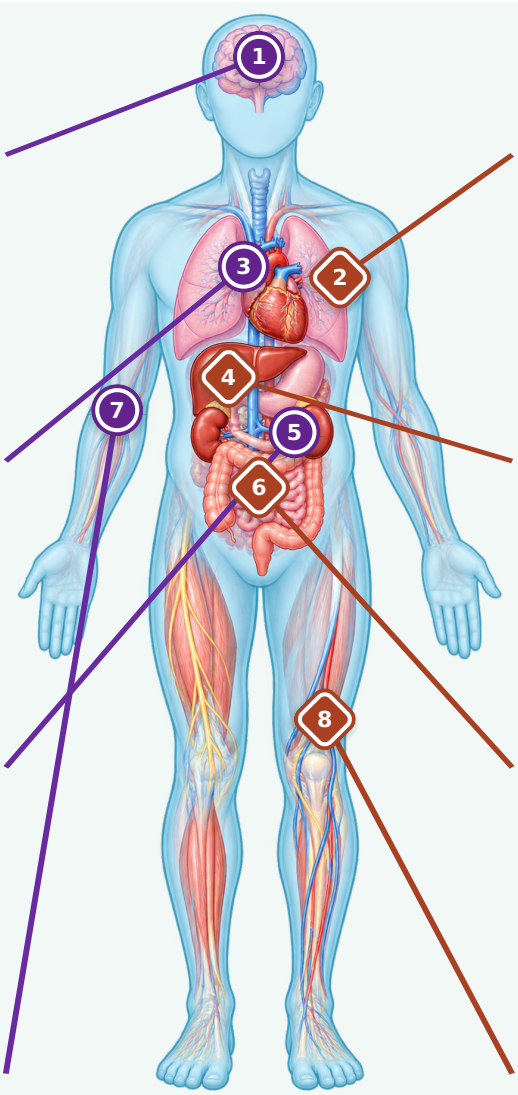
Nivolumab / ipilimumab

Kidney inflammation may be detected on blood tests. Report reduced urine, swelling, blood in urine or a marked change from normal.

7 Skin / mouth

Nivolumab / ipilimumab

Report rash or itching early. Blistering, peeling, mouth or genital sores, or rash with fever needs urgent assessment.



2 Lungs / breathing

Nivolumab / ipilimumab

New cough or breathlessness may reflect lung inflammation or another serious cause. Call promptly; severe breathing difficulty is an emergency.

4 Liver

Nivolumab / ipilimumab

Inflammation may first show in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

6 Bowels / abdomen

Nivolumab / ipilimumab

Report increased stool frequency, diarrhoea, blood or mucus, or abdominal pain early. Severe pain, black stools or dehydration needs urgent help.

8 Nerves / strength

Nivolumab / ipilimumab

New tingling, weakness, unsteadiness, facial change or trouble swallowing needs prompt review. Sudden weakness or speech difficulty is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Nivolumab + Ipilimumab

Up to 4 combination visits, 3 weeks apart; then nivolumab alone every 2 or 4 weeks as prescribed.

Phase: _____ Planned cycles: 4

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1 — Date / initials

C2 — Date / initials

C3 — Date / initials

C4 — Date / initials

My treatment information

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.